

Question:

A 5-year-old boy is brought to the physician by his mother because he does not “listen to her” anymore. The mother also reports that her son cannot concentrate on any tasks lasting longer than just a few minutes. Teachers at his preschool report that the patient is more active compared to other preschoolers, frequently interrupts or bothers other children, and is very forgetful. Last year the patient was expelled from another preschool for hitting his teacher and his classmates when he did not get what he wanted and for being disruptive during classes. He was born at term via vaginal delivery and has been healthy except for 3 episodes of acute otitis media at the age of 2 years. He has met all developmental milestones. His mother has major depressive disorder and his father has Graves' disease. He appears healthy and well nourished. Examination shows that the patient does not seem to listen when spoken to directly. The remainder of the examination shows no abnormalities. Which of the following is the most appropriate next step in treatment? Answer Choices: A. Behavior therapy B. Methimazole C. Fluoxetine D. Hearing aids

Reasoning + Answer:

<think>The patient had a history of attention problems, hyperactivity, and disruptive behavior. The mother had major depressive disorder and the father had Graves' disease. The patient had otitis media. Behavior therapy, methimazole, and fluoxetine were potential treatments. Hearing aids were mentioned as a possible intervention for attention problems. </think><answer> Behavior therapy</answer>

